

ENCOUNTER 4 OF 5

Date of Service: 2024-12-28 | Type: OUTPATIENT VISIT | Specialty: Orthopedic Surgery | Setting: Outpatient | Acuity: LOW

Attending Provider:	Dr. Anita Reyes, MD	Location:	New Amandaville Orthopedic & Sports Medicine Clinic
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Chief Complaint

Post-operative wound check, 7 days status-post left foot ganglion cyst excision and tarsal tunnel decompression.

Interval History

Ms. Gardner returns 7 days after surgical excision of a ganglion cyst compressing the left posterior tibial nerve. She reports the surgical site pain has been well controlled with acetaminophen and a short course of prescribed oxycodone-acetaminophen, which she used sparingly and has since discontinued as pain has improved. She denies fever, chills, increasing redness, drainage, or wound dehiscence. She reports the pre-operative pins-and-needles sensation in her toes has substantially improved, though she continues to note mild residual numbness immediately around the surgical incision, which she was counseled to expect. She has been compliant with activity restrictions, keeping the leg elevated and avoiding heavy lifting or bending. She has been using the provided post-operative surgical shoe for ambulation and denies falls.

She reports mild residual swelling of the left foot and ankle, improving daily, and denies any new numbness, weakness, or symptoms in the contralateral extremity. She has not yet returned to her regular work duties and is scheduled for a graduated return-to-work plan pending today's evaluation.

Review of Systems (12-point)

System	Findings
Constitutional	Denies fever, chills, or unintentional weight loss.
HEENT	History of seasonal allergies; denies vision changes or hearing loss.
Cardiovascular	History of hypertension, well controlled; denies chest pain or palpitations.
Respiratory	History of mild intermittent asthma; denies current dyspnea or wheeze.
Gastrointestinal	Reports occasional constipation; denies nausea, vomiting, or GI bleeding.
Genitourinary	Reports occasional urinary frequency; denies dysuria or hematuria.
Musculoskeletal	Reports mild left knee stiffness; denies acute joint swelling elsewhere.
Neurological	Reports numbness/tingling in left toes; denies headache, seizure, or syncope.
Psychiatric	History of generalized anxiety, stable on no pharmacotherapy; denies SI/HI.
Endocrine	History of hypothyroidism, on levothyroxine; denies heat/cold intolerance.
Integumentary	Reports occasional dry skin; denies rash or new lesions.
Hematologic/Lymphatic	History of mild anemia; denies easy bruising or lymphadenopathy.

Past Medical History

Chronic conditions include essential hypertension (dx. 2016), mild intermittent asthma (dx. childhood), and primary hypothyroidism (dx. 2019, on levothyroxine replacement). Surgical history is notable for a low-transverse cesarean section (2010) and a tonsillectomy (childhood). The patient was hospitalized previously for community-acquired pneumonia (2015) and following a motor vehicle collision (2018) with no long-term sequelae reported at that time.

Family History

Mother: hypertension, type 2 diabetes mellitus, living. Father: coronary artery disease, myocardial infarction at age 61, living. One sibling (sister): hypothyroidism. No known family history of malignancy, bleeding disorders, or hereditary neuropathy.

Social History

The patient is a non-smoker with no history of tobacco use and denies use of illicit substances. She reports occasional social alcohol use (1-2 drinks per week) and regular caffeine intake (1-2 cups of coffee daily). She works as an administrative coordinator, a role that requires prolonged standing and walking throughout the day. She is married and lives with her spouse and two children. She reports a generally sedentary exercise pattern with occasional weekend walking.

Current Medications

Medication	Dose	Route	Frequency	Indication
Aspirin	81 mg	PO	Daily	Cardioprotective
Lisinopril	10 mg	PO	Daily	Hypertension
Albuterol HFA	2.5 mg (nebulized equiv.)	Inhaled	PRN	Asthma
Levothyroxine	50 mcg	PO	Daily	Hypothyroidism
Acetaminophen	1000 mg	PO	PRN	Pain

Allergies

No known drug allergies (NKDA). No known food or environmental allergies of clinical significance documented.

Vital Signs

Parameter	Value	Parameter	Value
Blood Pressure	122/68 mmHg	Heart Rate	66 bpm
Temperature	97.8 F	Respiratory Rate	16 breaths/min
Pain Score	2/10	SpO2	98% RA
Weight	153 lbs	Height	5 ft 5 in
BMI	25.5 kg/m2		

Physical Examination

General

Well-appearing female in no acute distress, ambulating with a post-operative surgical shoe on the left foot.

Wound Examination

The medial ankle incision is well-approximated with sutures dissolving/absorbing as expected, without evidence of dehiscence, erythema, induration, or purulent drainage. Mild expected surgical edema is present around the incision. No fluctuance or signs of hematoma or seroma. Steri-strips remain intact over a small portion of the incision and were left in place.

Neurovascular Exam

Toes are warm, well-perfused, with capillary refill less than 2 seconds. Sensation to light touch is improved compared to the pre-operative exam throughout the forefoot and toes, with expected mild residual numbness immediately adjacent to the incision itself. Motor function of the toes and ankle is intact, 5/5 strength with dorsiflexion, plantarflexion, inversion, and eversion, limited only mildly by post-surgical discomfort.

Musculoskeletal

Mild residual swelling of the left ankle and foot, improving. Left knee stiffness unchanged from baseline, full range of motion.

Pathology Result Review

Test	Result	Reference Range	Flag
Surgical Pathology - Left foot mass	Benign ganglion cyst, no evidence of malignancy	N/A	Normal

Final surgical pathology on the excised specimen confirmed a benign ganglion cyst with no atypical or malignant features, consistent with the intra-operative impression.

Assessment

- Status post left foot ganglion cyst excision with tarsal tunnel decompression, post-operative day 7, healing well with improving neurologic symptoms and no signs of surgical site infection.
- Hypertension, controlled.
- Mild intermittent asthma, controlled.
- Primary hypothyroidism, controlled.

Plan

- Remove remaining steri-strips in clinic today; incision may be left open to air.
- Discontinue post-operative surgical shoe; transition to regular supportive footwear as tolerated.
- Begin gentle range-of-motion and gait-retraining physical therapy, 2x weekly for 3 weeks.
- Continue home medications without change.
- Graduated return to work duties over the next 1-2 weeks, avoiding prolonged standing greater than 30 minutes initially.
- Follow up with neurology in 8 weeks to reassess residual sensory symptoms and consider repeat nerve conduction study if incomplete resolution.

7. Return sooner for fever, increasing redness or drainage from the incision, new or worsening weakness, or loss of sensation.

Patient Education / Instructions

The patient was reassured regarding the benign pathology results and expected trajectory of continued neurologic recovery over the coming weeks to months as the nerve heals following decompression. She was counseled on gradual return to activity, wound care now that steri-strips have been removed, and signs/symptoms that would warrant urgent evaluation. She verbalized understanding and had no further questions at this time.

Care Team

Name	Role	Contact
Anita Reyes, MD	Attending Orthopedic Surgeon	555-201-4410
Karen Ibbotson, RN	Clinic Nurse	555-201-4420
Foothill Physical Therapy	Outpatient Physical Therapy	555-201-9010

Pain & Functional Assessment Questionnaire

Question	Response
Average pain over past 3 days (0-10 scale)	2/10
Worst pain over past 3 days (0-10 scale)	4/10 (with dressing change)
Pain medication use	Acetaminophen only, last opioid dose POD #3
Sleep quality	Improved, minimal nighttime pain
Weight-bearing status	Full weight-bearing in surgical shoe
Overall satisfaction with recovery so far (0-10)	8/10

Discharge Medication Reconciliation

Medication	Dose	Route	Frequency	Indication
Aspirin	81 mg	PO	Daily	Cardioprotective
Lisinopril	10 mg	PO	Daily	Hypertension
Albuterol HFA	2.5 mg (nebulized equiv.)	Inhaled	PRN	Asthma
Levothyroxine	50 mcg	PO	Daily	Hypothyroidism
Acetaminophen	1000 mg	PO	PRN	Pain

Medication reconciliation performed today confirms no changes to the patient's chronic home medication regimen. The short course of oxycodone-acetaminophen prescribed at the time of surgery has been completed and was not refilled, as the patient's pain is now well controlled with acetaminophen alone. The patient was counseled on safe disposal of any remaining opioid tablets at a local pharmacy take-back location.

Care Coordination Note

Physical therapy referral for post-operative gait retraining and range-of-motion work was submitted to Foothill Physical Therapy, with the first appointment scheduled within the week. A work status note was provided to the patient's employer outlining a graduated return-to-work plan with initial restriction to alternate sitting/standing and no prolonged standing greater than 30 minutes for the first two weeks. The patient's primary care physician was updated on surgical outcome and post-operative course. The patient was reminded to complete her scheduled neurology follow-up appointment in approximately 8 weeks.

Electronically signed by: **Anita Reyes, MD**
Department: Orthopedic Surgery
Signed: 2024-12-28 10:05 AM